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/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 75 of 117



A 49-year-old female 6 days post-transplant develops mental status changes. She had an uneventful transplant and was extubated on post-operative day 2. On the morning of day five she started complaining of headache and was noted to have visual hallucinations. At the time she was afebrile with a blood pressure of 147/96. AM Labs were notable for a tacrolimus trough level of 14 ng/ml, a WBC of 5.3 10⁹/L and a creatinine of 4mg/dl. That evening she had a grand mal seizure and needed urgent intubation.

The next best diagnostic test for this patient is:

- A. Spinal tap
- ☒ B. Magnetic Resonance Imaging of the brain
- C. Serum quantitative cytomegalovirus polymerase chain reaction test
- D. Non contrast CT scan of the brain

Commentary

References

Testing Point: The Board-certified AHFTC specialist should recognize PRES as a cause of mental status changes in the immediate peri-transplant period.

Answer: B. Magnetic Resonance Imaging of the brain.

MRI of the Brain. Given the patient's symptoms and elevated blood pressure, tacrolimus and creatinine she most likely has posterior reversible encephalopathy syndrome (PRES). PRES is characterized by a headache, seizures, altered mental status and visual loss. MRI findings can be diagnostic and are notable for white matter vasogenic edema affecting the posterior occipital and parietal lobes of the brain predominantly.

Answer A: Though infection is possible, the clinical scenario and laboratory findings are more consistent with PRES.

Answer C: Though CMV is in the differential, it is less likely given the constellation of clinical findings. Leaving PRES untreated may lead to seizures and possibly death.

Answer D: Though non-contrast CT would identify intracranial hemorrhage, it would not identify PRES.

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